2.2 Post-operative Care

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain near the wound is severe." • Gives his pain 8/10 **Objective** • Guards the incision • Grimacing • HR 110/min, BP 145/90 mmHg	**ACTUAL** **Acute pain** related to surgical tissue trauma as evidenced by pain score 8/10, guarding and tachycardia.	Pain down to 3/10 or less within 1 hour of analgesia. Able to deep breathe and get up early, with pain he can tolerate.	• Score pain on a numeric scale every 2–4 hours and again after each intervention. • Give analgesics on time; if he is on opioids, keep an eye on sedation and respiratory rate. • Help him into a comfortable position, and have him hold a pillow over the wound when he coughs. • Time activities so they come after his analgesia. • Add non-drug measures as well, such as relaxation and a quiet room.	Pain 3/10 or less when rechecked: **goal met**. Pain still not controlled? Go over the analgesic plan with the physician.
Subjective • "I cannot cough because it hurts." **Objective** • Coarse crackles at both bases • Cough weak, not clearing anything • SpO_2 92% on room air • RR 24/min	**ACTUAL** **Ineffective airway clearance** related to effects of anesthesia and pain on coughing as evidenced by crackles, weak cough and reduced SpO_2.	Chest clear on auscultation and SpO_2 95% or more within 24 hours.	• Listen to the chest and check SpO_2 every 4 hours. • Raise the head of the bed to 30–45 degrees. • Every 1–2 hours, get him to breathe deeply and cough with the incision splinted. • While he is awake he should use the incentive spirometer 10 times an hour. • Get him out of bed early, as ordered. • Give oxygen as prescribed.	Chest clear, SpO_2 96%: **goal met**. Secretions still there? Think about chest physiotherapy and let the physician know.
Subjective • Feels dizzy when he sits up **Objective** • Abdominal surgery 4 hours ago • Wound drain in situ • On prophylactic anticoagulant	**RISK** **Risk for bleeding** as evidenced by risk factors: recent surgery, surgical drain and anticoagulant therapy.	Vital signs stable, dressing dry, drain output within the expected range.	• Vital signs every 15 minutes for the first hour, then as per policy. • Look at the dressing and the drain; mark the level and record what drains. • Follow Hb and coagulation results. • Report HR over 100/min, a falling BP, or a drain that is filling fast. • Stay with him the first time he walks, because postural hypotension can make him fall.	Vital signs stable and very little drainage: **goal met**. Active bleeding goes to the surgeon immediately.